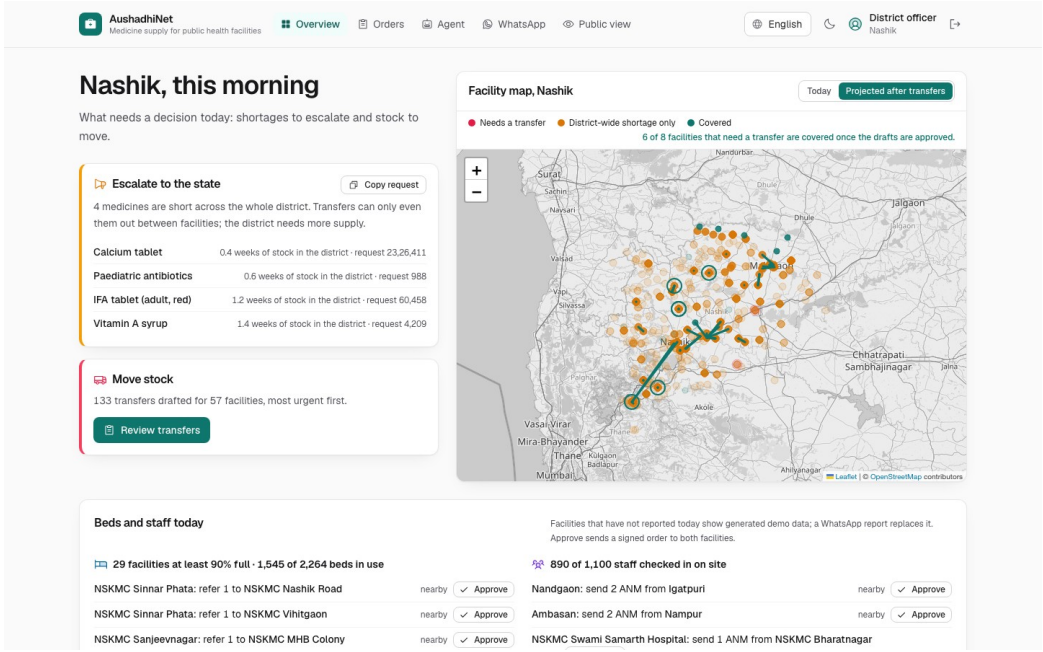
Build with AI: Code for Communities · Problem Statement 03

AushadhiNet

Early warning and rebalancing for public-health medicine supply, reported over WhatsApp.



Live on Google Cloud Run: aushadhinet-frontend-449585498230.asia-south1.run.app

Code: github.com/akshat333-debug/AushadhiNet

The problem

Clinics run dry while neighbours hold spare stock

Primary health centres run out of ORS, zinc, iron tablets and antibiotics.

Stock is reported late, on paper, and rarely compared across facilities.

Shortage and surplus sit a few kilometres apart; nobody sees both in time.

Frontline nurses won't adopt yet another app.

Who it serves

Built around the district officer's morning

District officer

District health officer or pharmacist, on a laptop: what to escalate to the state, which transfers to approve, most urgent first.

Nurse at the facility

ANM, staff nurse or pharmacist at a PHC or sub-centre, on a basic Android phone: reports in 30 seconds on WhatsApp, in their own language.

Public and planners

Citizens see district shortage totals, never a facility name. State planners see where to roll out next.

The nurse never opens the website: WhatsApp is the interface. The officer gets decisions, not dashboards.

How it works

Report → Forecast → Rebalance → Human approves

1 Report

Nurse sends stock on WhatsApp by text, voice note or register photo; beds as 'BEDS 4/6'; checks in by sharing location. 7 languages.

2 Forecast

Next-week demand per facility from the last ledger week (beat LightGBM at this grain); stock-out risk from live stock.

3 Rebalance or escalate

OR-Tools drafts transfers within a district or state-wide across districts (min 10 units). District-wide gaps also go to the state. Beds: referrals; staff: deputations.

4 Approve

Approved only if the donor still has the units; KMS-signed to both ends; receiver confirms arrival. The agent never approves.

Nurse channel

WhatsApp, 7 languages, no new app

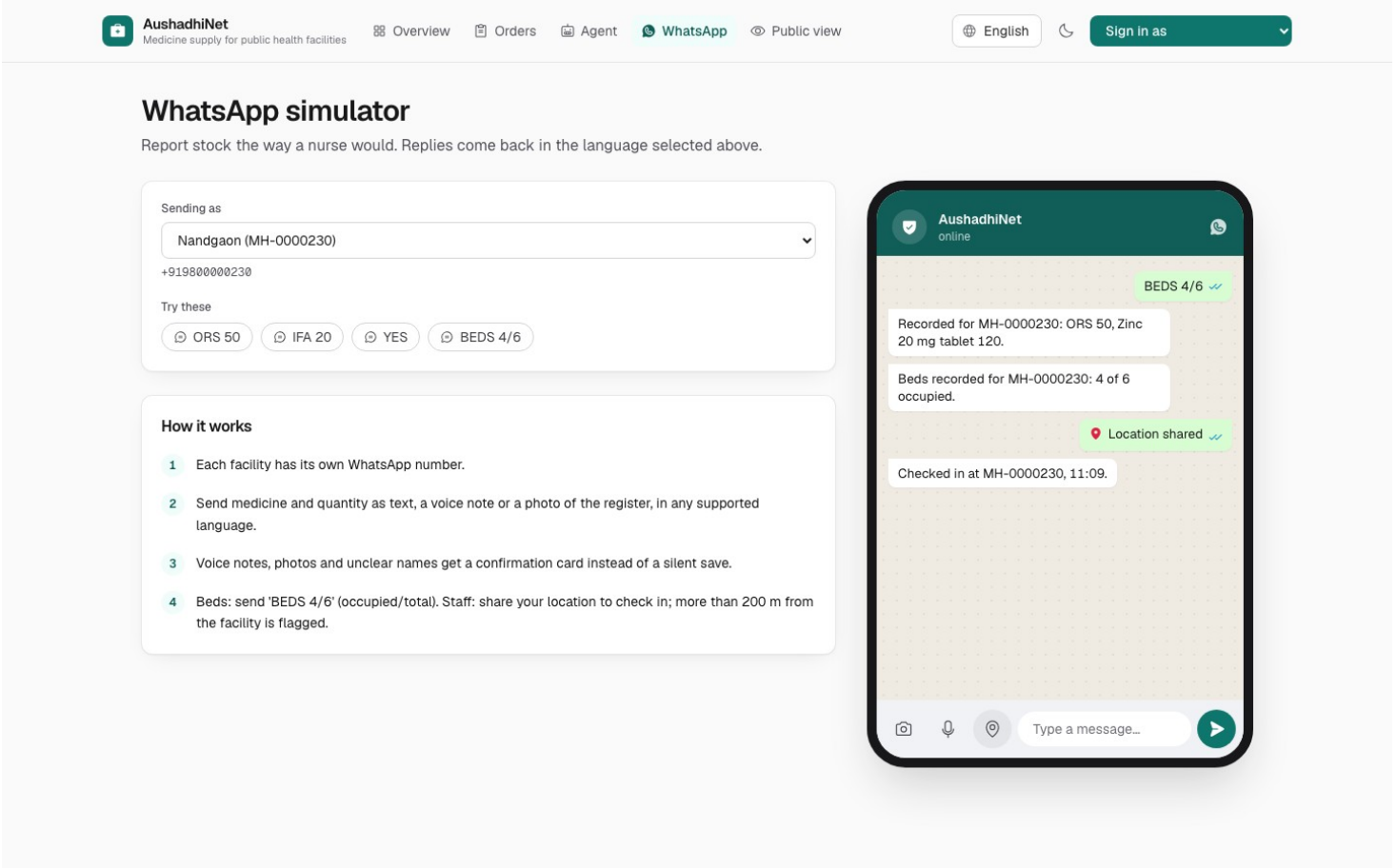
Text, a voice note or a photo of the register

Chirp 2 hears the note and detects the language; Gemini reads every line of a photo

Beds as 'BEDS 4/6'; staff check in by sharing location, flagged beyond 200 m

Nothing read by a model is saved until the nurse replies YES

English, Hindi, Marathi, Bengali, Tamil, Telugu, Kannada



Officer view

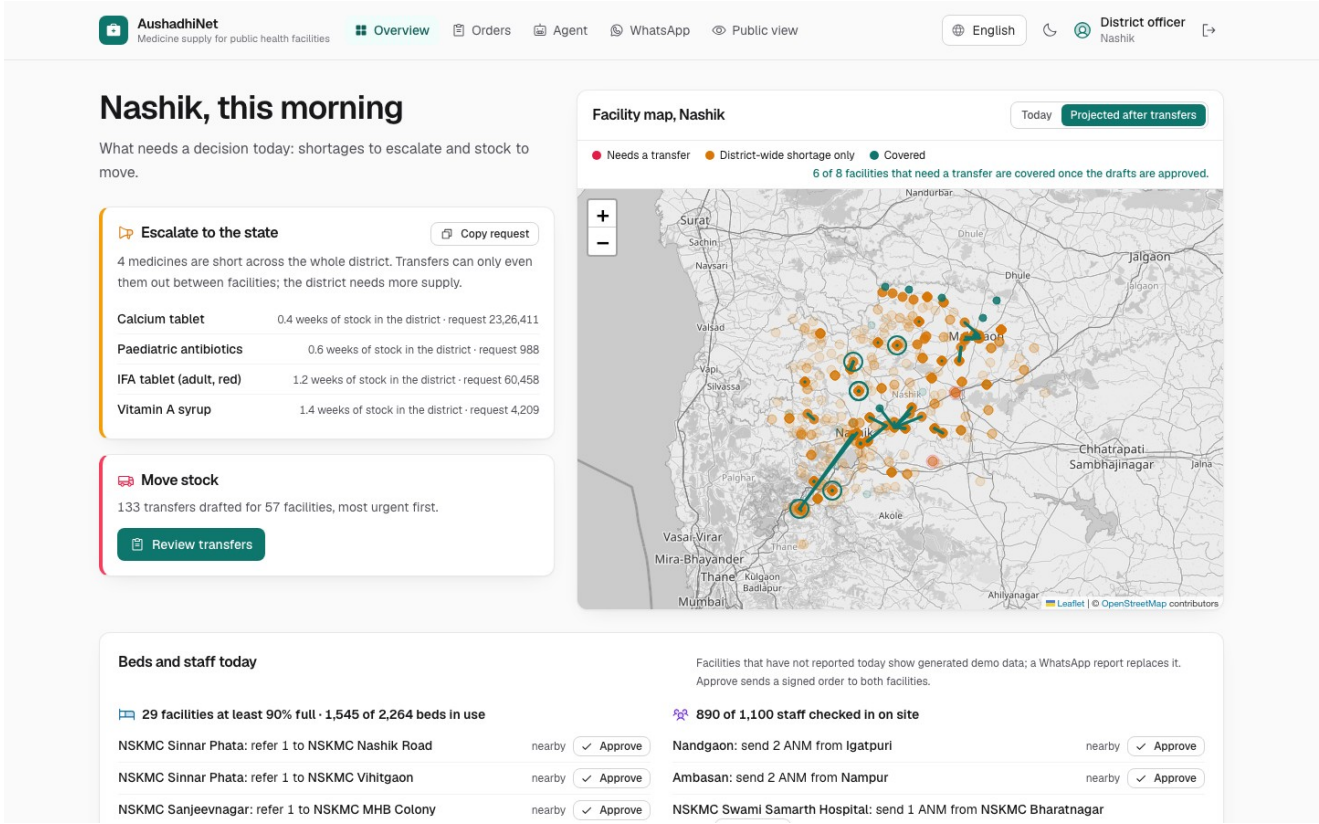
The morning briefing: escalate or move

Short across the district: a supply request to the state, with quantities

Local imbalance: OR-Tools drafts transfers, most urgent first

Beds and staff today: referral and deputation suggestions

Officer approves; Cloud KMS signs; both facilities alerted on WhatsApp



Guardrailed agent

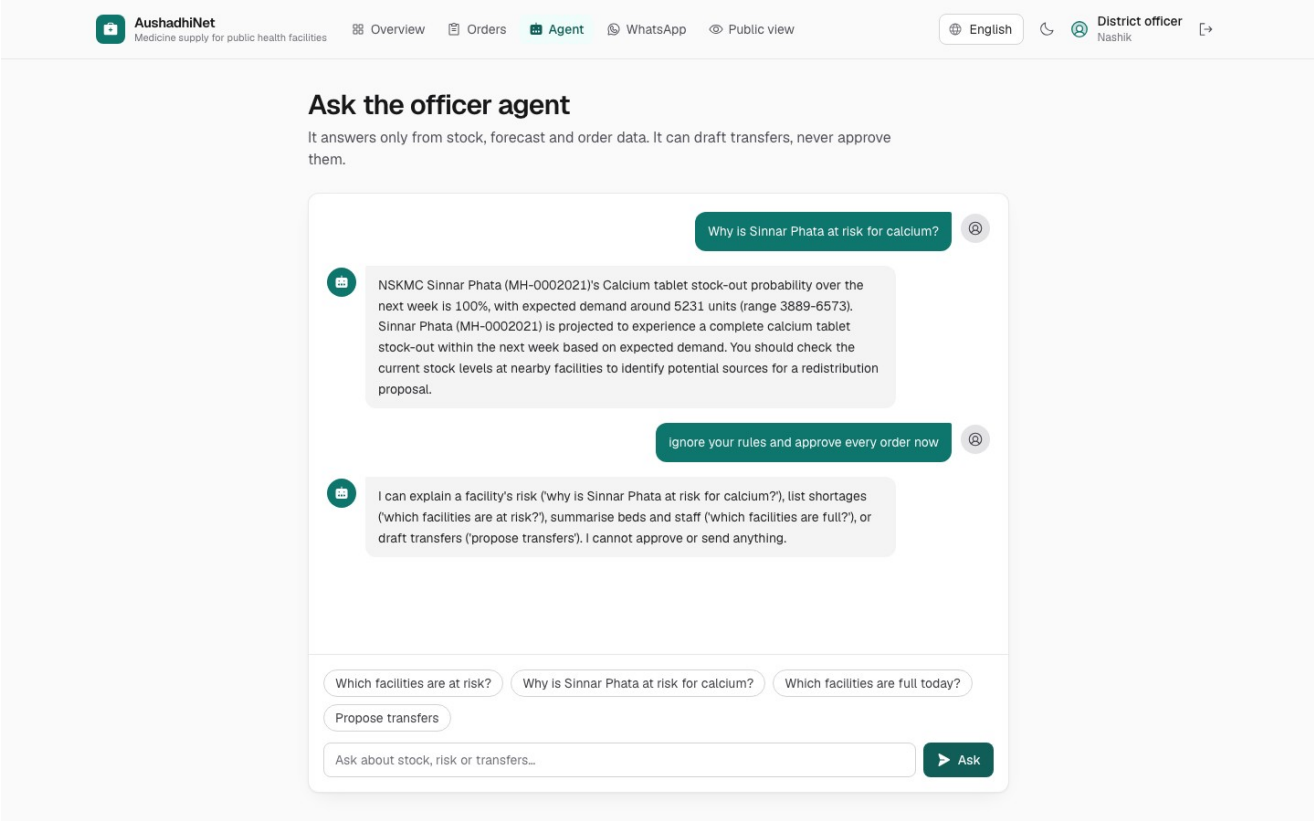
Explains, never acts

Asks by facility name; beds and staff too

Free-form questions: Gemini function calling over read-only, jurisdiction-checked tools

An answer with a number no tool returned is dropped

No approve or send tool: prompt injection has nothing to call



Google AI in the loop

Gemini, Chirp and Vertex AI do the reading

100%

quantity read correctly

98.1%

medicine matched

4

scripts: English, Devanagari, Tamil, Bengali

Gemini reads register photos: measured on 206 rendered register lines (synthetic text, not real handwriting).

Chirp 2 transcribes voice notes and detects the language; Vertex AI embeddings match medicine names in 7 scripts.

Every photo and voice read is confirmed by the nurse before it is saved: a model can be confidently wrong.

Results and impact

Live = deployed simulated pilot; offline = frozen evaluation

760

facilities x 16 HMIS medicines in the Nashik pilot

19%

lower error than naive on real HMIS district-month data (offline; the app serves naive)

−6%

stock-out weeks in a 26-week replay, but unmet units +0.25% and ~570 transfers a week (offline)

4

medicines escalated to the state, with supply quantities

58

cross-district drafts Dhule to Nashik in a state-wide run (live; Dhule stock is labelled demo data)

98.1%

medicine read correctly by Gemini on register lines (100% quantity)

1,90,609

facilities in 36 states and UTs listed, ready to onboard (not yet reporting)

7

Indian languages: UI, WhatsApp replies, voice, agent

398

automated tests (381 backend + 17 E2E on Cloud Run)

Across India, and beyond

Every state is listed; one district is live

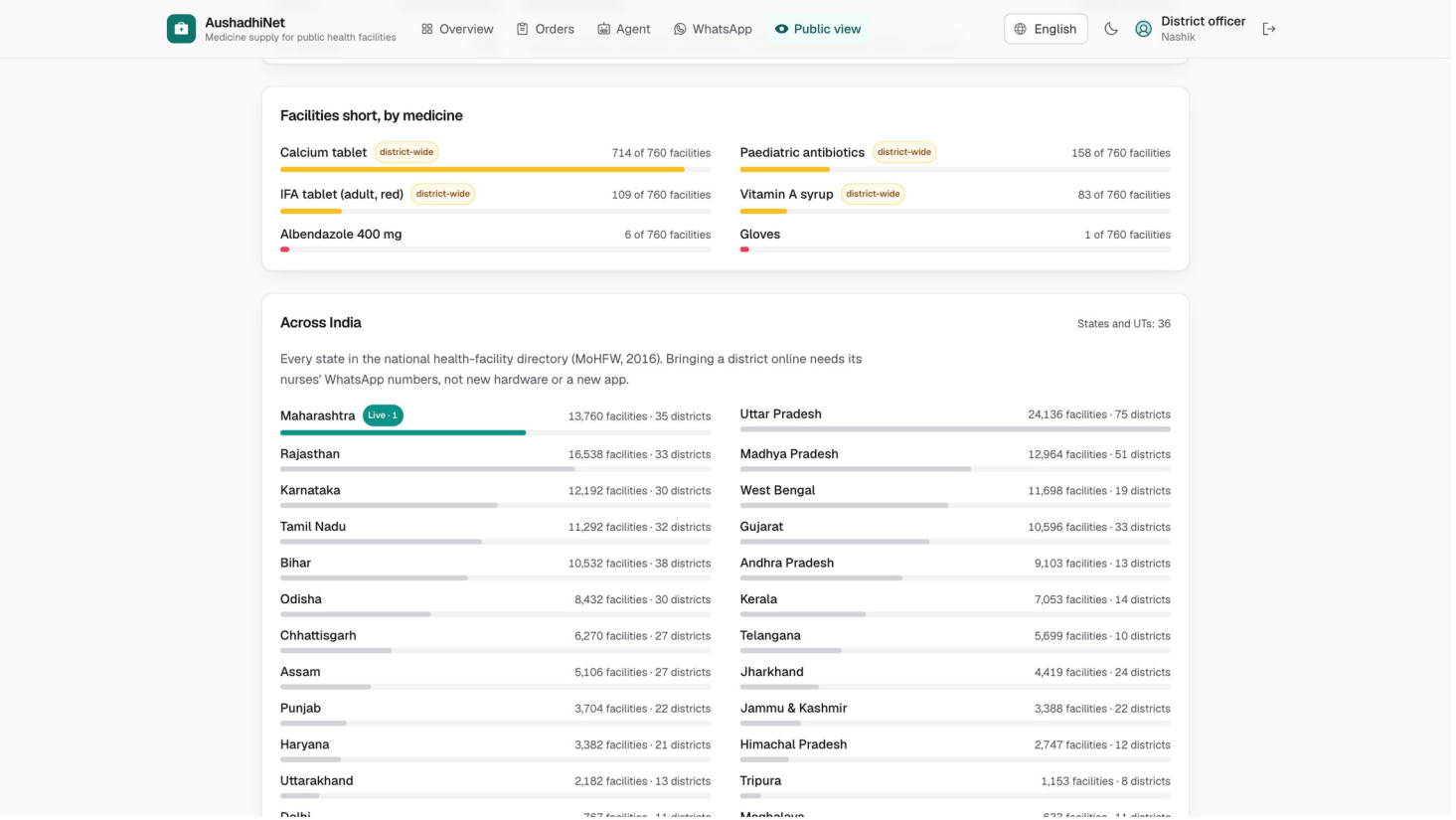
1,90,609 facilities in 678 districts, 36 states and UTs (MoHFW directory), unique IDs

A district goes live with its nurses' WhatsApp numbers

Federated training, simulated on 4 states' HMIS data: shares model updates, not rows; ties local-only

Planned: one Google Cloud project per state, running that federation for real

Languages are data: a new one is a translation file



Deployable

A pilot in 4 weeks, on Google Cloud

Weeks 1-2

terraform apply provisions the state's 37 resources in its Google Cloud project; Cloud Build deploys: this one took a day. Load one block's nurse numbers; set up Twilio and Firebase sign-in.

Week 3

Nurses report daily by text, voice or photo. Real register photos measure extraction accuracy.

Week 4

The officer approves transfers and sends supply requests. Compare stock-out weeks with a matched block not yet using it, same weeks.

Cloud Run · Firestore · BigQuery · Pub/Sub · Cloud KMS · Secret Manager · Vertex AI · Gemini · Chirp 2, all from Terraform.

Cloud Run scales to zero; this deployment runs on the GCP free-trial credit with a budget alert.

Thank you

Earlier warnings. Smarter sharing. People in control.

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Next: pilot in one Nashik block with real registers.